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Prasanna A V
55 Year(s) · Female

Reported on
December 05, 2025

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Prasanna A V

55 Year(s)/Female

Ref. by :

Partner : -

Report Ref. ID : BLR5854178

Patient ID : OHP2R3JD1796497

Collected : 05/12/2025 06:36 AM

Received : 05/12/2025 08:44 AM

Reported : 05/12/2025 10:21 AM



MC-6367



Test	Results	Units	Biological Reference
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BIOCHEMISTRY**Glucose, Random**

Fluoride Plasma, Glucose Oxidase-Peroxidase (GOD-POD)

85

mg/dL

70 - 140

Clinical Significance:

Blood Sugar also known as Blood glucose test is commonly performed to diagnose and monitor Diabetes Mellitus. In common practice the blood sugar is measured and reported as 3 different tests based on the meal intake status of an individual.

Low blood glucose level may be due to Overdose Insulin, Insulinomas, Starvation, Adrenal insufficiency, Drinking excessive alcohol, Severe liver disease, Hypopituitarism, Hypothyroidism, Severe infections.

High blood glucose level most frequently indicate diabetes, but many other diseases and conditions can also cause elevated blood glucose. e.g. Acromegaly, Acute stress (response to trauma, heart attack, and stroke for instance), Cushing syndrome, Hyperthyroidism, Pancreatic cancer, Pancreatitis.

The reference interval has been referred from the American diabetes Association (<https://www.diabetes.org/a1c/diagnosis>). Kindly consider the reference range of blood sugar according to the fasting status or post meal status.

Name of Test	Meal Status	Reference Range
Fasting Blood Sugar	Overnight Fasting	70-99
Random Blood Sugar	Sample taken irrespective of meal status	70-140
Post Prandial Sugar	Sample taken after 2 Hrs of meal consumption	70-140

Glycosylated Haemoglobin (HbA1c) | Whole Blood EDTA**Glycated Hemoglobin (HbA1C)**

High-Performance Liquid Chromatography (HPLC)

5.1

%

Normal: < 5.7
Pre-Diabetes: 5.7-6.4
Diabetes: ≥ 6.5

Mean Blood Glucose

Calculated

100

mg/dL

< 117

HbA1C is used to monitor fluctuations in blood glucose concentration in the past 8 to 12 week's period.

The reference interval defined as per American Diabetes Association guidelines 2016:

- Less than 5.7%: Non Diabetic
- 5.7 to 6.4%: at increased risk of developing diabetes in the future
- More than 6.5%: Diabetic
- Therapeutic glycemic target
 - Adults: less than 7%
 - Children with Type 1 diabetes: less than 7%
- Pregnant diabetic patients: less than 6.5%

Note: Targets may be individualized based on: Age/life expectancy, Comorbid conditions, Diabetes duration, Hypoglycemia status, Individual patient considerations

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Reference: American Diabetes Association. Standards of medical care in diabetes - 2021.

Mean Blood Glucose is average Blood glucose which directly correlates with A1C, reported in the same units as blood sugar levels (mg/dl). Thus it reflects the average glucose concentration in the past 8 to 12 weeks period. This should not be compared with Fasting or Post prandial or random blood sugar which measures glucose concentration at that point of time of testing.

Lipid Profile | Serum**Cholesterol, Total**Cholesterol Esterase/Cholesterol
Oxidase/Peroxidase**251**

mg/dL

< 200

Triglycerides

Cholesterol Oxidase

151

mg/dL

< 150

High-Density Lipoprotein (HDL)**Cholesterol**Cholesterol Esterase/Cholesterol
Oxidase/Peroxidase**45**

mg/dL

> 50

**Non-High Density Lipoprotein
(Non-HDL) Cholesterol**

Calculated

206

mg/dL

< 130

Low-Density Lipoprotein (LDL)**Cholesterol**

Calculated

175

mg/dL

< 100

**Very Low-Density Lipoprotein
(VLDL) Cholesterol**

Calculated

30

mg/dL

< 30

**Cholesterol/High Density
Lipoprotein (HDL) Ratio**

Calculated

5.5

3.3 - 4.4

**Low-Density Lipoprotein/High-
Density Lipoprotein (LDL/HDL)
Ratio**

Calculated

3.9

0.5 - 3

**High-Density Lipoprotein/Low-
Density Lipoprotein (HDL/LDL)
Ratio**

Calculated

0.3

> 0.4

Remarks	Total Cholesterol (mg/dL)	Triglycerides (mg/dL)	LDL Cholesterol (mg/dL)
Optimal	<200	<150	<100

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Above Optimal	-	-	100-129
Borderline	200-239	150-199	130-159
High	≥ 240	200-499	160-189
Very High	-	≥ 500	≥ 190

Lipid profile is a group test consisting of various lipids. Lipid profiles are generally collected with overnight fasting. However, recent guidelines have recommended non fasting samples for lipid profile for assessment of cardiovascular risk. The details for the study can be checked at <https://jamanetwork.com/journals/jamainternalmedicine/fullarticle/2733560>

In certain instances measurements in the same patient can show physiological and analytical variations. In such cases three serial samples at an interval of 1 week each are recommended for Total cholesterol, TG, HDL and LDL.

Cholesterol levels are increased in primary hypercholesterolemia; secondary hyperlipoproteinemia, including nephrotic syndrome; primary biliary cirrhosis; hypothyroidism; and in some cases, diabetes mellitus. Low cholesterol levels may be found in malnutrition, malabsorption, advanced malignancy, and hyperthyroidism.

Triglyceride levels are used in the diagnosis and treatment of patients with diabetes mellitus, nephrosis, liver obstruction, other diseases involving lipid metabolism, or various endocrine disorders.

High Density Lipoprotein (HDL) cholesterol levels is used to evaluate the risk of developing coronary heart disease (CHD). The risk of CHD increases with lower HDL cholesterol concentrations.

LDL (low-density lipoprotein) cholesterol level, sometimes called "bad" cholesterol, makes up most of our body's cholesterol. High levels of LDL cholesterol raise your risk for heart disease and stroke.

Very-low-density lipoprotein (VLDL) cholesterol is produced in the liver and released into the bloodstream to supply body tissues with triglycerides. High levels of VLDL cholesterol have been associated with the development of plaque deposits on artery walls, which narrow the passage and restrict blood flow.

Liver Function Test (LFT) | Serum

Bilirubin, Total Diazo Method	1.08	mg/dL	0.2 - 1.3
Bilirubin, Direct Calculated	0.23	mg/dL	0 - 0.3
Bilirubin, Indirect Reflectance Spectrophotometry	0.86	mg/dL	0.1 - 1.1
Aspartate Aminotransferase (AST) Multipoint-Rate/UV with Pyridoxal-5- Phosphate (P-5-P)	23	U/L	14 - 36
Alanine Transaminase (ALT) LDH, UV Kinetic	21	U/L	<50

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Test	Results	Units	Biological Reference
Aspartate Aminotransferase/Alanine Transaminase (AST/ALT) Ratio Calculated	1.1		0.7 - 1.4
Alkaline Phosphatase (ALP) Multipoint-Rate/UV with Pyridoxal-5- Phosphate (P-5-P)	94	U/L	38 - 126
Gamma-Glutamyl Transpeptidase (GGT) SZAZ Carboxylated Substrate	16	U/L	12 - 43
Protein Biuret	7.10	g/dL	6 - 8.3
Albumin Bromo-Cresol Green	4.08	g/dL	3.5 - 5
Globulin Calculated	3.02	g/dL	2.3 - 3.5
Albumin/Globulin (A/G) Ratio Calculated	1.4		0.8 - 2

In certain individuals, **total bilirubin** up to 2.0 mg/dl is considered normal. High bilirubin values can be due to jaundice.

Total bilirubin is invariably increased in jaundice. Causes of jaundice are prehepatic, resulting from various hemolytic diseases; hepatic, resulting from hepatocellular injury or obstruction; and posthepatic, resulting from obstruction of the hepatic or common bile ducts.

Increased **direct bilirubin** levels can occur in hepatobiliary disorders, including intrahepatic and extrahepatic biliary tree obstruction, liver cell damage, Dubin-Johnson syndrome, and Rotor syndrome.

High **indirect bilirubin** levels can occur in hemolytic disorders, Gilbert's syndrome, Crigler-Najjar syndrome, neonatal jaundice, and ineffective erythropoiesis.

High **Aspartate Aminotransferase** values can occur in Myocardial infarction, pulmonary emboli, skeletal muscle trauma, alcoholic cirrhosis, viral hepatitis, or drug-induced hepatitis.

Elevated **Alanine Aminotransferase** levels are seen in liver cell necrosis, hepatitis, hepatic cirrhosis, liver tumours, obstructive jaundice, Reye's syndrome, extensive trauma to skeletal muscle, myositis, myocarditis, or myocardial infarction.

High **alkaline phosphatase** levels can be due to primary and secondary hyperparathyroidism, Paget's disease of bone, carcinoma metastatic to the bone, osteogenic sarcoma, Hodgkin's disease, Hepatobiliary diseases involving cholestasis, inflammation, or cirrhosis.

ALP levels can also be elevated in fever and increased bone metabolism(e.g., in adolescents and during the healing of a fracture), in renal infarction and failure and in pregnancy complications.

Low **ALP** levels may occasionally be seen in hypothyroidism.

Gamma-glutamyl transferase (GGT) is a sensitive indicator of hepatobiliary disease. It is useful in the diagnosis of obstructive jaundice and chronic alcoholic liver disease, in the follow-up of chronic alcoholics undergoing treatment, and in the detection of hepatotoxicity. GGT is more responsive to biliary obstruction than AST, ALT, or ALP.

Total **protein** levels can be used to evaluate nutritional status.

High **protein** concentrations can be due to dehydration, Waldenström's macroglobulinemia, multiple myeloma, hyperglobulinemia,

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Test	Results	Units	Biological Reference
granulomatous, and some tropical diseases. Low protein concentrations can be due to pregnancy, excessive intravenous fluid administration, cirrhosis or other liver diseases, chronic alcoholism, heart failure, nephrotic syndrome, glomerulonephritis, neoplasia, protein-losing enteropathies, malabsorption, and severe malnutrition. Increased albumin levels may indicate dehydration or hyperinfusion with albumin. Decreased albumin levels are found in rapid or over-hydration, severe malnutrition and malabsorption, severe diffuse liver necrosis, chronic active hepatitis, and neoplasia. Albumin is commonly reduced in chronic alcoholism, pregnancy, renal protein loss, thyroid dysfunction, peptic ulcer disease, and chronic inflammatory diseases. Globulin includes carrier proteins, enzymes, complement, and immunoglobulins. Most of these are synthesised in the liver, although immunoglobulins are synthesised by plasma cells. Increased globulin level usually results from an increase in immunoglobulins. Malnutrition and congenital immune deficiency can decrease globulin levels due to decreased synthesis. Nephrotic syndrome can cause decreased globulin levels due to protein loss through the kidney. AST/ALT Ratio > 2:1 (AST is two times higher than ALT) is indicative of alcoholic liver disease. AST/ALT Ratio < 1:1 (ALT is higher than AST) indicates non-alcoholic fatty liver disease.			

Kidney Function Test with Electrolytes (KFT / RFT) | Serum

Urea Urease	29	mg/dL	15 - 36
Creatinine Twopoint-Rate-Creatinine Aminohydrolase	0.73	mg/dL	0.52 - 1.04
Blood Urea Nitrogen (BUN) Calculated	13.5	mg/dL	6 - 20
Blood Urea Nitrogen (BUN)/Creatinine Ratio Calculated	18.49	Ratio	10 - 20
Uric Acid Uricase	3.8	mg/dL	2.5 - 6.2
Calcium Arsenazo Method	9.2	mg/dL	8.4 - 10.2
Phosphorus Phosphomolybdate Formation	4.2	mg/dL	2.5 - 4.5
Estimated Glomerular Filtration Rate (eGFR) Twopoint-Rate-Creatinine Aminohydrolase/Calculation	97	ml/min/1.73m ²	Normal: ⇒ 90 Mild decrease: 60-89 Mild moderate decrease: 45-59 Severe decrease: 15-29 End stage kidney disease: < 15

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Test	Results	Units	Biological Reference
Electrolytes			
Sodium Direct ISE	142	mmol/L	137 - 145
Potassium Direct ISE	5.4	mmol/L	3.5 - 5.5
Chloride Direct ISE	103	mmol/L	98 - 107

High **sodium** levels can be caused by dehydration, disorder of the adrenal glands, diarrhoea, diuretics, kidney disease, and diabetes insipidus.

Low **sodium** levels are caused by diarrhoea, vomiting, kidney disease, liver failure, Addison's disease, and malnutrition.

High **potassium** levels may be due to low aldosterone, kidney failure, metabolic or respiratory acidosis.

Low **potassium** levels may be due to diarrhoea, vomiting, diuretics, and high aldosterone.

Chloride is used to help diagnose conditions related to an imbalance of acids or fluids in the body. Certain medicines, such as antacids, can also cause abnormal results.

High **chloride** levels may be due to Addison's disease, metabolic acidosis, respiratory alkalosis, and renal tubular acidosis.

Low **chloride** levels may be due to burns, CHF, metabolic alkalosis, vomiting, dehydration, and respiratory acidosis(compensated).

High **urea** and **BUN** levels are suggestive of poor kidney function due to acute or chronic kidney diseases, decreased blood flow to the kidneys as in congestive heart failure, shock, stress, recent heart attack or severe burns, bleeding from the gastrointestinal tract, conditions that obstruct urine flow or dehydration.

Low **urea** and **BUN** levels are uncommon and are not usually a cause for concern. They can be seen in severe liver disease or malnutrition but are not used to diagnose or monitor these conditions. Low urea levels are also seen in normal pregnancy.

Creatinine is elevated in kidney disease, damage, infection, urinary tract obstruction, reduced blood flow to the kidneys in case of shock, congestive heart failure, complications of diabetes.

High levels of **uric acid** are seen in kidney disease, pre-eclampsia, purine-rich food, alcoholism, and side effects of cancer treatment.

Low **calcium** levels may be due to hypoparathyroidism, kidney failure, pancreatitis, malnutrition, or a disorder in calcium absorption.

High **calcium** levels may be due to hyperparathyroidism, hyperthyroidism, sarcoidosis, drugs like diuretics, and excessive calcium supplementation.

High **phosphorus** levels can be due to dehydration, hypoparathyroidism, hypervitaminosis D, metastases to bone, sarcoidosis, pulmonary embolism, renal failure, or diabetes mellitus with ketosis.

Low **phosphorus** levels can be caused by hyperparathyroidism, high calcium levels, sepsis, vitamin D deficiency, renal tubular disorders, chronic hemodialysis, vomiting, or occasionally decreased dietary phosphate intake.

Chronic Kidney Disease often has no symptoms until the later stages. So, reliable estimates of GFR are important for identifying the disease as early as possible.

Factors that can affect **eGFR** include pregnancy, being over the age of 70, unusual muscle mass, cirrhosis, nephrotic syndrome, a past solid organ transplant, and some medications.

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Test	Results	Units	Biological Reference
HAEMATOLOGY			
Complete Blood Count (CBC) with ESR Whole Blood EDTA			
Red Blood Cells (RBC) Count DC Impedance Method	4.41	mill/mm ³	3.8 - 4.8
Hemoglobin (Hb) Cyanide-free SLS method	13.2	g/dL	12 - 15
Hematocrit (HCT) Packed Cell Volume (PCV) Calculated	39.6	%	36 - 46
Mean Corpuscular Volume (MCV) Calculated	89.7	fL	83 - 101
Mean Corpuscular Hemoglobin (MCH) Calculated	29.9	pg	27 - 32
Mean Corpuscular Hemoglobin Concentration (MCHC) Calculated	33.3	g/dL	31.5 - 34.5
Red Cell Distribution Width (RDW) CV Calculated	12.8	%	11.6 - 14
Mentzer Index Calculated	20.3		Beta Thalassemia trait: < 14 Iron deficiency anaemia: >= 14
Sehgal Index Calculated	1824.5		Beta Thalassemia trait: < 972 Iron deficiency anaemia: >= 972
Total White Blood Cell Count (TC) Flow Cytometry	5600	cells/mm ³	4000 - 10000
Differential Count			
Neutrophils Flow Cytometry	57.7	%	40 - 80
Lymphocytes Flow Cytometry	33.8	%	20 - 40
Monocytes Flow Cytometry	5.9	%	2 - 10

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Test	Results	Units	Biological Reference
Eosinophils Flow Cytometry	2.2	%	1 - 6
Basophils Flow Cytometry	0.4	%	0 - 2
Absolute Neutrophil Count (ANC) Calculated	3231	/mm ³	2000 - 7000
Absolute Lymphocyte Count (ALC) Calculated	1893	/mm ³	1000 - 3000
Absolute Monocyte Count (AMC) Calculated	330	/mm ³	200 - 1000
Absolute Eosinophil Count (AEC) Calculated	123	/mm ³	20 - 500
Absolute Basophil Count (ABC) Calculated	22	/mm ³	0 - 100
Neutrophil Lymphocyte Ratio (NLR) Calculated	1.7		1 - 3
Platelet Count DC Impedance Method	234	10 ³ /μL	150 - 450
Platelet Hematocrit Calculated	0.222	%	0.2 - 0.5
Mean Platelet Volume (MPV) Calculated	9.5	fL	7 - 13
Erythrocyte Sedimentation Rate (ESR) Quantitative Capillary Photometry	19	mm/h	0 - 20

- Reference Ranges are in accordance with Dacie & Lewis Practical Hematology International Edition (12th)
- As per International Council for Standardization in Hematology's recommendations Differential Leucocyte counts are additionally reported in Absolute numbers in each cell per unit volume of blood.

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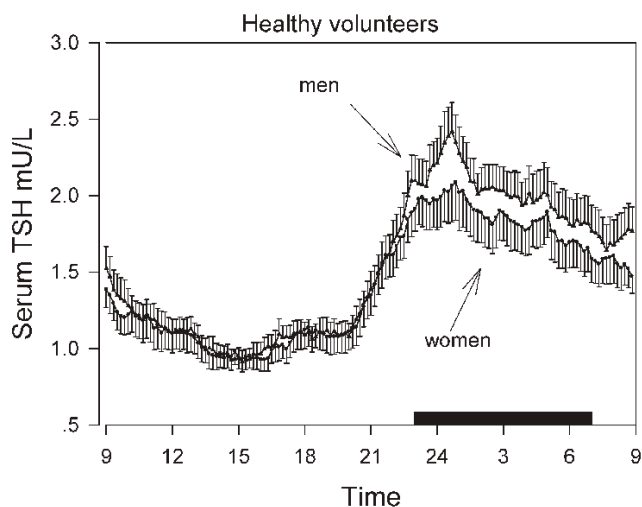
Test	Results	Units	Biological Reference
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IMMUNOLOGY

Thyroid Function Test (TFT) | Serum

Triiodothyronine (T3), Total Chemiluminescent Immunoassay	1.04	ng/mL	0.62 - 2.79
Thyroxine (T4), Total Chemiluminescent Immunoassay	10.5	µg/dL	5.53 - 11
Thyroid Stimulating Hormone (TSH) Chemiluminescent Immunoassay	1.342	µIU/mL	0.5 - 8.9

Thyroid Stimulating Hormone (TSH), also called Thyrotropin is a hormone secreted into the blood by the Pituitary gland (a gland present in the brain). It signals the thyroid gland to make and release the thyroid hormones (T3 & T4) into the blood. High TSH level indicates that the thyroid gland is not making enough thyroid hormone (primary hypothyroidism). Low TSH level usually indicates that the thyroid is producing too much thyroid hormone (hyperthyroidism).



Factors influencing TSH levels

- TSH level** shows a significant decline after meal intake in comparison to fasting values. If the patient is taking any thyroid medication different times each day, they may sometimes be taking the thyroid hormone on an empty stomach, and sometimes with or after having food. This may have clinical implications in the diagnosis and management of hypothyroidism, especially Subclinical hypothyroidism.
- Circadian Rhythm:** TSH levels follow a circadian variation, reaching peak levels between Morning 2 - 4 am and at a minimum between Evening 6-10 pm. The above graph considers a sleep window of 11:00 PM to 7:30 AM. The variation is of the order of 50%. There are studies which quote variations up to 70 % depending on when the sample is drawn during which time of the day. Hence time of sample collection during a day can significantly influence on the measured serum TSH concentrations.
- Other Factors:** It is important to recognize that TSH is a labile hormone and is subject to non-thyroidal pituitary influences (glucocorticoids, somatostatin, dopamine etc.), stress, activity, that can disrupt the TSH/FT4 relationship. Genetics, Poisonous substances and radiation exposure, Inflammation of the thyroid gland, Deficiency or excess of iodine in the diet, Pregnancy, Certain medications – antidepressants, cholesterol lowering drugs, chemotherapy drugs, steroids, Thyroid cancer.

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Test

Results

Units

Biological Reference

In **pregnant females** the reference range of TSH differs. Please refer the table below for the same:-

Pregnancy	TSH Reference Range (μU/mL)
1st Trimester	0.100-2.500
2nd Trimester	0.200-3.00
3rd Trimester	0.300-3.00

References:

- Indian Journal of Endocrinology and Metabolism 18(5):p 705-707, Sep-Oct 2014.
- http://www.pnei-it.com/1/upload/thyrotropin_secretion_patterns_in_health_and_disease.pdf

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Test	Results	Units	Biological Reference
Vitamin B12, Cyanocobalamin Serum, Chemiluminescent Immunoassay	107	pg/mL	107.2 - 653.3

Clinical Significance:

Decreased Levels: Lack of Intrinsic factor, Total or partial gastrectomy, Atrophic gastritis, Intrinsic factor antibodies, **Malabsorption:** Regional ileitis, resected bowel, Tropical Sprue, Celiac disease, pancreatic insufficiency, bacterial overgrowth & achlorhydria, Loss of ingested vitamin B12, fish tapeworm, **Dietary deficiency:** Vegetarians, **Congenital disorders:** Orotic aciduria & transcobalamin deficiency, **Increased demand:** Pregnancy especially last trimester. **Increased Levels:** Chronic renal failure, Congestive heart failure, Acute & Chronic Myeloid Leukemia, Polycythemia vera, Carcinomas with liver metastasis, Liver disease, Drug induced cholestasis & Protein malnutrition. **Note:** An associated increase in homocysteine levels is an independent risk marker for cardiovascular disease and deep vein thrombosis. HoloTranscobalamin II levels is a more accurate marker of active Vit B12 component.

Vitamin D, 25-Hydroxy
Serum, Chemiluminescent Immunoassay

20.7

ng/mL

Deficient: < 10
Insufficient: 10-30
Sufficient: 30-100
Potential Toxicity: > 100

Clinical Significance:

For the diagnosis of vitamin D deficiency, it is recommended that there be a clinical correlation with serum 25(OH)vitamin D, calcium, parathyroid hormone, and alkaline phosphatase. While monitoring oral vitamin D therapy, serum 25(OH)vitamin D should be tested after 3 months of treatment. However, the required dosage of vitamin D supplements and time to achieve target vitamin D levels show seasonal and individual variability depending on age, body fat, sun exposure, physical activity, genetic factors (especially variable vitamin D receptor responses), associated liver or renal disease, malabsorption syndromes, and calcium or magnesium deficiency influencing the metabolism of vitamin D.

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Test	Results	Units	Biological Reference
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CLINICAL PATHOLOGY

Urine Complete Analysis | Urine

Urine, Physical Examination

Volume Manual	50	mL	
Colour Manual	Pale yellow		Pale yellow
Appearance RGB Sensor Technology	Clear		Clear

Urine, Chemical Examination

pH Double Indicator Method	5.0		5 - 8
Specific Gravity Bromo Thymol Blue Indicato	1.015		1.001 - 1.035
Protein Protein Error of pH Indicator	Nil		Nil
Glucose Enzyme Method Glucose Oxidase- Peroxidase (GOD-POD)	Nil		Nil
Ketones Nitroprusside Method/Dipstick	Nil		Nil
Bilirubin Azo Coupling Method	Nil		Nil
Blood Peroxidase Activity	Negative		Negative
Urobilinogen Azo Coupling Method	Normal		Normal
Leucocyte Esterase Granulocyte Esterase Method	Negative		Negative
Nitrites Griess Method	Negative		Negative

Urine, Microscopic Examination

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Test	Results	Units	Biological Reference
Pus Cells Manual Microscopy	1-2	/hpf	0-5
Epithelial Cells Manual Microscopy	0-1	/hpf	0-2
Red Blood Cells (RBC) Manual Microscopy	Nil		0-2
Granular Casts Manual Microscopy	Nil	/hpf	Nil
Hyaline Casts Manual Microscopy	Nil	/hpf	Occasional
Uric Acid Crystals Manual Microscopy	Nil	/hpf	Nil
Phosphate Crystals Manual Microscopy	Nil	/hpf	Nil
Calcium Oxalate Crystals Manual Microscopy	Nil	/hpf	Nil
Amorphous Urates Manual Microscopy	Nil	/hpf	Nil
Amorphous Phosphates Manual Microscopy	Nil	/hpf	Negative
Yeast Automated Morphological Microscopy	Nil		Nil
Bacteria Manual Microscopy	Nil		Nil
Parasites Manual Microscopy	Nil	/hpf	Nil
Mucus Manual Microscopy	Absent		Absent

Increased protein in urine is seen in dehydration, kidney disorders, heart failure and transplant rejection. 24 hour urine protein and Protein/creatinine ratio in a random urine sample recommended if increased.

Glucosuria can be seen in kidney disorders, uncontrolled diabetes mellitus, hormonal disorders, and pregnancy. To be correlated with plasma glucose levels.

Ketonuria is seen in physical exercise, starvation, severe vomiting, exposure to cold, uncontrolled diabetes (diabetic ketoacidosis)

Increased bilirubin levels should be followed up with Liver function tests and indicates conjugated hyperbilirubinemia.

Increased urobilinogen can be seen due to haemolysis, megaloblastic anaemia and haemorrhage in tissues. Urobilinogen is absent or reduced in obstructive liver disease and antibiotic therapy.

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Test

Results

Units

Biological Reference

RBCs in urine (Haematuria) can be seen in anticoagulant therapy, bleeding diathesis and traumatic catheterization history to be looked into. Dysmorphic RBCs suggestive of glomerular pathology. Non glomerular diseases line calculus, infections, tumours, after strenuous exercise and diseases of the prostate.

Increase in pus cells are elevated in cases of UTI, to be correlated with urine culture, if clinically indicated. Infection can be in either the upper or lower urinary tract or with acute glomerulonephritis, tubule interstitial nephritis. Leucocyte esterase detects esterase enzyme released from the granules of leucocytes.

Infected urine may contain considerable amounts of nitrite as a result of bacterial nitrate reductase activity, and detection of nitrite in urine is routinely used in the diagnosis of bacterial cystitis. It is indicative of the requirement of Urine culture and sensitivity testing for identification and treatment of UTI.

Hyaline casts are seen normally (not associated with disease states) seen after strenuous exercise and with non renal diseases, such as dehydration.

Granular casts can be seen in acute glomerulonephritis and pyelonephritis.

Dr. Sayantani Sarkar
MBBS, MD (Pathology)
Pathologist

Dr. Drisya Jagadees
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Pathologist

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CONDITIONS OF LABORATORY TESTING & REPORTING

- It is presumed that the test sample belongs to the patient named or identified in the test requisition form. Test results released pertain to the specimen submitted.
- Laboratory investigations are only a tool to facilitate arriving at a diagnosis and should be clinically correlated by the Referring Physician.
- All tests are performed and reported as per the turnaround time stated in the Orange Health Labs Directory of Services (DOS).
- Orange Health Labs confirms that all tests have been performed or assayed with the highest quality standards, clinical safety & technical integrity.
- All test results are dependent on the quality of the sample received by the Laboratory and the assay technology.
- Report delivery may be delayed due to unforeseen circumstances. Inconvenience is regretted.
- A requested test might not be performed if:
 - The specimen received is insufficient or inappropriate, or the specimen quality is unsatisfactory
 - Incorrect specimen type
 - Request for testing is withdrawn by the ordering doctor or patient
 - There is a discrepancy between the label on the specimen container and the name on the test requisition form
- Test results may show interlaboratory variations.
- Test results are not valid for medico-legal purposes.
- This is a computer-generated medical diagnostic report that has been validated by an Authorized Medical Practitioner/Doctor. The report does not need a physical signature.

REPORT PRINTOUT/HARDCOPY

Prasanna A V

55 Year(s)/Female

Ref. by :

Partner : -

Report Ref. ID : BLR5854178

Patient ID : OHP2R3JD1796497

Collected : 05/12/2025 06:36 AM

Received : 05/12/2025 08:51 AM

Reported : 05/12/2025 10:17 AM



MC-6367



Test

Results

Units

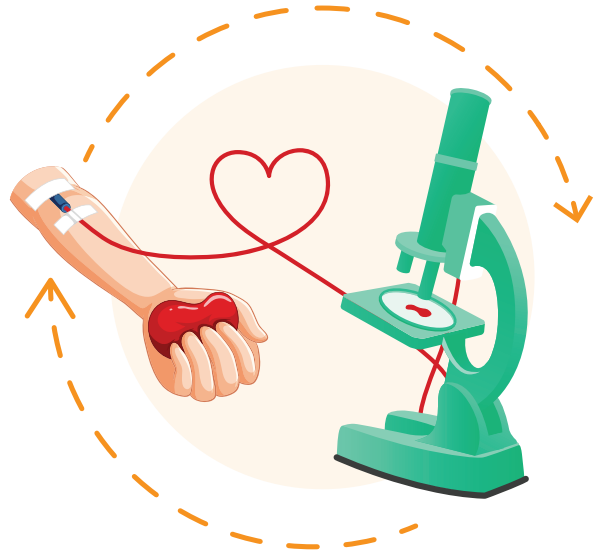
Biological Reference

- Patients can collect free report printouts from our Lab or the nearest Collection Centre.
- Hard copies can be delivered within 24 hours for ₹200/report (non-refundable after dispatch).
- For assistance, contact Orange Support at +91-9008111144.

IN MATTERS RELATED TO HEALTH, FASTER IS BETTER.

We equate showing care to showing urgency.

Blood starts deteriorating the minute it leaves the human vein, unless stored right. We have built our logistics to achieve the **fastest vein-to-machine testing time** in the industry, and in our journey ahead will strive to reduce it further as much as possible.



HOW ARE WE FASTER THAN OTHERS?

We identified two gaps in diagnostics: **sample transportation time** and **processing delays**.

We have solved for these problems by our approach of:

LAB EXCELLENCE

Our tests are processed in fully automated, cutting-edge in-house labs for end-to-end quality control.

NO BATCH TESTING

We don't batch your samples to save costs. They are tested the moment they come in.

NO TIME WASTED

Since we test your blood the fastest, you get the most accurate reports in the industry.